

Patient Encounter Documentation: SOAP Note Case Study

Fictional student submission. All patient data is simulated for coursework.

Part 1 — SOAP Note

Date: 10/14 · **Time roomed:** 10:15 · **Provider:** Dr. Okafor · **Visit type:** 3-month follow-up, DM type 2 and HTN · **Allergies:** PCN (rash)

S: 52 y/o male presents for scheduled 3-month follow-up for type 2 diabetes mellitus and hypertension. Pt states *"I've been getting these headaches in the morning, maybe three or four days a week for the last month."* Also reports *"my feet feel kind of numb, like pins and needles, mostly at night."* Pt reports he ran out of his blood pressure medication approximately 2 weeks ago and has not refilled it. Reports checking blood glucose "sometimes, maybe twice a week." Rates headache 4/10 at time of visit. Denies chest pain or SOB. Current meds per pt: metformin 1000 mg BID, lisinopril 20 mg daily (not taken x 2 weeks), atorvastatin 20 mg QHS.

O: Ht 5 ft 10 in. Wt 232 lb. BMI 33.3, which is in the obese range. T 98.4 °F oral. P 88 regular. R 18. SpO2 97% RA. BP 158/96 left arm, seated, adult cuff. Repeat BP 154/94 after 5 minutes. Random capillary BS 218 mg/dL by point-of-care meter. Pt alert and oriented, ambulates without assistance, no acute distress noted.

A: Blood pressure is elevated secondary to the patient not taking his lisinopril for the past two weeks. Numbness and tingling in the feet is likely early diabetic neuropathy. Glucose is uncontrolled and the patient is obese, which is making both conditions worse.

P: Provider notified of BP 154/94 and BS 218 prior to provider entering the room. Reviewed importance of taking blood pressure medication as prescribed and refilling before running out. Instructed pt to check blood glucose daily and bring his meter log to the next visit. Advised pt to eat healthy and get more exercise. Foot care handout given. Pt verbalized understanding. Pt to be seen by Dr. Okafor this visit; follow-up per provider.

Marisol Vega, MA student — 10/14, 10:32

Word count: 371

Part 2 — Documentation Rationale

The first choice I made was to quote the patient directly on his headaches and the numbness in his feet instead of summarizing them. If I had written "patient complains of chronic headaches and neuropathy," I would have added meaning that the patient never said. The headache pattern he described, three or four mornings a week for a month, is the part the provider needs, and my summary would have lost the timing. The words are his, so they belong in quotation marks in the subjective section.

The second choice was to document both blood pressure readings instead of just the lower one. My first reading was 158/96 and the repeat five minutes later was 154/94. It would look better on the chart to record only the second one, but that would misrepresent the encounter. I also recorded the arm, the position and the cuff size on the first reading so that whoever takes the next reading can repeat it the same way, since a different arm or a wrong cuff size can change the number by ten points or more.

The third choice was what I left out. The patient told me he works night shifts and sleeps about five hours a day. I did not put that in my note because I was not sure whether it belonged in the subjective section or whether it counted as social history that the provider would ask about anyway. Looking at it now I think leaving it out was probably the wrong call, since poor sleep and night shift work relate to both his blood pressure and his morning headaches, but I want to be honest about why it is not there.

Word count: 296

Part 3 — Patient Education Note

Reviewed with patient that his blood pressure medication only works while he is taking it, and that going two weeks without it is the most likely reason his readings today are higher than his last visit. Asked the patient what got in the way of refilling it. He said he did not realize he was out until the bottle was empty and his pharmacy is closed by the time his shift ends. Gave him the pharmacy's mail-order refill number and showed him how to set a refill reminder on his phone.

Reviewed daily glucose checking. Patient currently checks about twice a week. Asked him to check every morning before eating and to write the number down or keep it in the meter, and to bring the meter to his next visit so the provider can see the pattern.

Reviewed daily foot checks because of the numbness he described: look at the tops, bottoms and between the toes every day for cuts, blisters or color changes, and call the clinic for anything that is not healing. Gave the diabetic foot care handout.

Escalated to provider before the patient left the room: blood pressure 154/96 on repeat, random glucose 218 mg/dL, two weeks off lisinopril, and new numbness and tingling in both feet with morning headaches. I reported these to Dr. Okafor before he entered the room rather than waiting for him to read the chart.

Word count: 243

Part 4 — Scope of Practice and Privacy Reflection

A statement a medical assistant may not record as an assessment in this case is *"glucose is uncontrolled."* That is a clinical judgment about how the patient's diabetes is being managed over time, and only the provider can make it. I should have written it as an objective finding instead: *"Random capillary blood glucose 218 mg/dL at 10:22, point-of-care meter, non-fasting."* That records exactly what I measured and when, and it leaves the interpretation to Dr. Okafor.

A HIPAA consideration specific to this encounter is that the patient asked me to call his wife about his results because he goes straight to work after the appointment. I cannot release his information to her unless he has a signed authorization on file naming her. I documented the request and told the patient the front desk can help him complete the release form before he leaves.

Word count: 158

Sources

Booth, K. A., Whicker, L. G., & Wyman, T. D. (2022). *Medical Assisting: Administrative and Clinical Procedures* (7th ed.), Chapter 12, Patient Records and Documentation.

Centers for Disease Control and Prevention (2024). *Diabetes and Your Feet*.

Clinic Policy 4.2, Blood Pressure Measurement and Repeat Readings, revised 2024.

AI use declaration

I used Grammarly to check spelling and grammar in Parts 2, 3 and 4. I did not use any AI tool to write the SOAP note or the reflection.